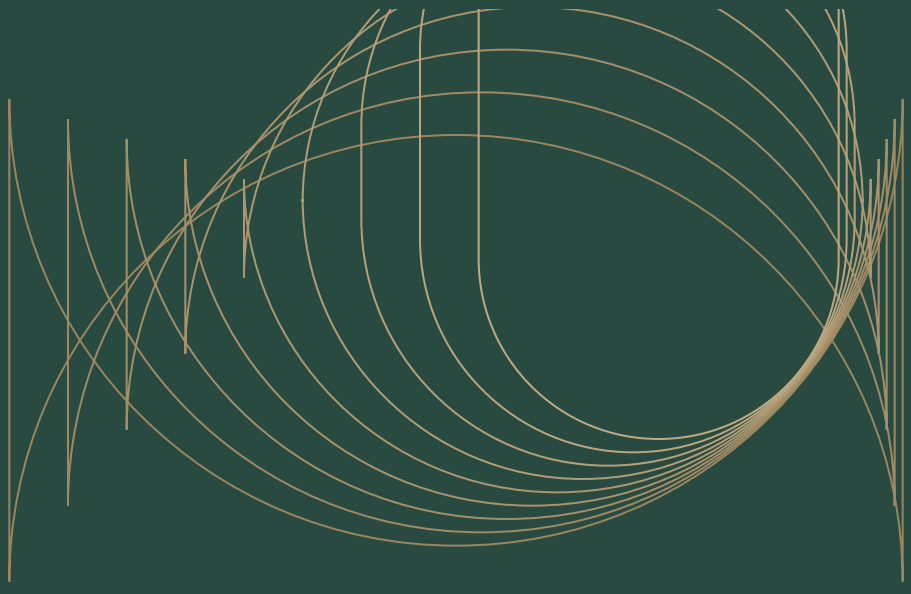
WELL MIND

GER BOLAND

ONE STEP FORWARD

Everyday Wellbeing

Sleep. Food. Movement. Attention.



AN EDUCATIONAL JOURNAL

Space to notice. Knowledge to use. A step that fits.

Daily check-in

A minute or two is enough. Fill in what is useful today; leave the rest blank.

Date

Sleep last night · hours

Mood today · /10

Energy today · /10

Mood: 0 = very low, 10 = very good. Energy: 0 = none, 10 = very high.

One thing on my mind

One thing I want to do for myself today

A little more, if useful

Yesterday’s numbers, only if already tracked. No need to calculate anything here.

Steps

Food energy · kcal

Protein · grams

Screen time · h/min

Movement yesterday

Something I did to care for myself

Use a fresh copy when you return. A blank box means “not recorded”, not zero. Need help? Find what you need · p. 4.

Your life right now

Give each area a score out of 10 based on how satisfied you feel with it right now. There are no right or wrong answers.

0 = not working for me; 10 = working very well. Rename an area or leave it blank.

Date

Rest & energy		/10
Food & body care		/10
Movement		/10
Relationships		/10
Work & money		/10
Home & support		/10
Play & creativity		/10
Meaning & self		/10

Example: Relationships 7/10 · Rest & energy 8/10 · Play & creativity 2/10.

This is a snapshot, not a test or a list of problems to fix. The lowest score does not have to come first.

An area I might give a little attention to

Keep these scores if you later explore the Wheel of Life · p. 34. You do not need to complete that chapter today.

Find what you need

Choose a question. **Do it** takes you to a practical page; **Understand it** offers more explanation. You can use either route.

I want to know…	Do it	Understand it
How am I today?	Daily check-in · p. 2	Weekly patterns · p. 6
How do I track steps?	Phone or no-device guide · p. 7	Movement and fitness · p. 37
How do I support sleep?	An evening routine · p. 9	Sleep & recovery · p. 8
How do I record food or protein?	Food logging guide · p. 24	Food choices · p. 14 Protein · p. 23
How do I estimate calorie needs?	Calculator and example · p. 21	Choose your approach · p. 19 Energy balance · p. 20
What could self-care look like?	My low-energy plan · p. 13	Everyday self-care · p. 10
How do I choose a realistic goal?	My first GROW · p. 36	Wheel of Life & GROW · p. 33
What do my life scores mean?	Behind the scores · p. 35	The full Wheel of Life · p. 34
Where do I feel discomfort?	My body map · p. 51	Mobility & comfort · p. 50
How do I find a little headspace?	One-minute pause · p. 31	Screen time & attention · p. 29

A simple rhythm

Check in when useful. Look back once a week. Explore one chapter when something catches your interest.

Tap the page links in this PDF. On paper, use the page numbers. External links and QR codes sit beside the relevant tools. Food and body measurements are optional.

Need a little support?

Reach out to Ger directly to arrange a coaching session and work through any of the material in this journal together.

Visit www.wellmind.ie

One Step Forward

Practical pages

Daily check-in · p. 2

Your life right now · p. 3

Find what you need · p. 4

My week in view · p. 6

Steps, at my own pace · p. 7

Explore when you are ready

01 Sleep & recovery · p. 8

02 Everyday self-care · p. 10

03 Food & body care · p. 14

04 Screen time & attention · p. 29

05 Meditation · p. 31

06 Wheel of Life & GROW · p. 33

07 Fitness checks · p. 37

08 Mobility & comfort · p. 50

09 Review & continue · p. 61

Sources · p. 63

No chapter is a prerequisite. “My next step” boxes use four plain questions: goal, now, options and next step. The optional GROW chapter explains this approach.

Type in answer spaces and tick checklists in a PDF app that saves forms. Keep a blank original; save a fresh copy for each entry. Use drawing tools for the wheel and body map. Check saved answers reopen.

For adult education and reflection, not diagnosis or treatment. Follow individual healthcare advice and the body-check stop guidance. A score does not identify the cause of a symptom.

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My week in view

Look for a pattern, not a perfect week. Use any notes you have, or simply think back. Choose the rows that matter to you.

Week beginning

Sleep / rest

Mood / energy

Movement

Food / protein, if useful

Self-care / support

A few words are enough: what helped, what made things harder, or an average if you already have it. No need to copy every daily number.

Something that went well

A challenge I experienced

My goal: what moved forward, and what did I learn?

One priority for next week - and a small step or support

If you use a separate weekly tracker, keep the daily figures there. This page is for reflection. Return to Your life right now · p. 3 occasionally, when a fresh snapshot would help.

Steps, at my own pace

A step count is optional. It can help you notice a pattern; it is not a score for the value of your day.

Do it

iPhone: Open Health and find Steps. If it is not in your Summary, add it to Pinned. Open the step chart and select the day you want to record.

Apple / Health setup and step data
Official instructions. A watch is not required to use iPhone step data.
support.apple.com/en-ie/104997

Android, if using Google Fit: Open Fit, tap Steps, then choose the day. Use the count shown for that date.

Google Fit / Find your activity
Official instructions for finding daily and weekly activity.
support.google.com/fit/answer/6090183

Understand it

Use the same device where possible, and note when you did not carry or wear it. Do not add phone and watch totals together. Numbers can be incomplete or inaccurate.

No tracker, or steps are not useful for you? Describe your movement, time outdoors, a seated activity or an everyday task instead. There is no universal step target in this journal.

What would I like movement to support in my everyday life?

Record yesterday's total on Daily check-in · p. 2. For optional task observations, see Fitness checks · p. 37.
Apps, menus and permissions can change; use the official help links above.

Sleep & recovery

Consider: What would better-supported rest make possible?

Sleep supports how we feel and function. Helpful habits include a fairly regular sleep schedule, a quieter wind-down, daylight and suitable activity during the day, and a dark, quiet sleeping space. Caffeine later in the day and alcohol near bedtime can interfere with sleep.

Shift work, caring, pain and health conditions can change what is realistic. Choose an action within your influence. You cannot command yourself to fall asleep.

Notice the opportunity

How much time have I left for sleep? What gets squeezed into that time?

Notice the experience

Do I feel restored? Am I waking often? What else might be affecting me?

What might I protect or ask for before buying another sleep product?

What would a great leader do?

Would they ask for more effort, or make recovery possible?

General sleep-habit guidance: NHLBI. Persistent sleep difficulty, marked daytime sleepiness or breathing concerns deserve a clinical conversation; journalling is not treatment for a sleep disorder.

An evening routine

CHAPTER REFLECTION

A possibility	My version
A more regular wake time, where realistic	
A short wind-down before bed	
Move a late caffeinated drink earlier	
Ask for practical help with evening demands	

Tonight I will try _____ because _____.

MY NEXT STEP

Goal	Is rest still the change I want to support? _____
Now	What helped, what was difficult, and what remains unclear? _____
Options	Keep, simplify, change or ask for support? _____
Next step	I will carry _____ into my next evening and review on _____.

The Matt Walker Podcast / episode 145
Optional listen: How to get More Deep Sleep. Matthew Walker is a sleep researcher, not a medical doctor.
thematthewalkerpodcast.buzzsprout.com/1821163/episodes/19553110-145-how-to-get-more-deep-sleep

Everyday self-care

Consider: What part of me or my day needs attention?

Self-care includes food, rest, healthcare, support, limits, connection and pleasure. Identify the need first; choose a response that fits your resources.

A kind of care	What it could mean for me
Body	Food, water, rest, comfort or appropriate healthcare.
Mind + feelings	Less stimulation, reflection, support, learning or a pause.
Relationships	Company, honesty, repair, distance or a clear request.
Practical life	Money advice, an easier routine, shared tasks or accessible support.
Meaning + play	Something enjoyable, absorbing or connected to what matters.

What do I keep calling a luxury that may actually be a need?

Care for body & mind

Body

- ☐ Eat regularly
- ☐ Choose food that supports me
- ☐ Move in a way that suits me
- ☐ Get enough rest
- ☐ Take breaks
- ☐ Spend time outdoors
- ☐ Reduce unhelpful screen use
- ☐ Wear comfortable clothes
- ☐ Attend to health needs
- ☐ Make room for physical enjoyment

Mind

- ☐ Reflect or journal
- ☐ Learn something new
- ☐ Read or listen to something meaningful
- ☐ Be curious
- ☐ Try an unfamiliar experience
- ☐ Reduce avoidable stress
- ☐ Notice thought patterns
- ☐ Allow unstructured mental space
- ☐ Make time for a hobby
- ☐ Ask for professional support when needed

What is already helping? What feels missing?

One small action and what would make it easier:

Care across everyday life

Feelings

- ☐ Spend time with people I enjoy
- ☐ Stay connected
- ☐ Talk honestly about feelings
- ☐ Acknowledge effort
- ☐ Allow feelings to be present
- ☐ Make room for laughter
- ☐ Seek comfort
- ☐ Set boundaries
- ☐ Use encouraging self-talk
- ☐ Repair after conflict

Work + practical life

- ☐ Take breaks
- ☐ Manage workload realistically
- ☐ Set limits
- ☐ Protect focused time
- ☐ Make the environment workable
- ☐ Ask for help
- ☐ Balance work and personal time
- ☐ Do meaningful tasks
- ☐ Connect with supportive colleagues
- ☐ Finish or release draining tasks

Meaning + spirit

- ☐ Spend time in nature
- ☐ Do something meaningful
- ☐ Connect with values or faith
- ☐ Notice gratitude without forcing it
- ☐ Welcome a new perspective
- ☐ Make room for stillness
- ☐ Contribute to a community or cause
- ☐ Create, pray, meditate or reflect
- ☐ Notice awe or connection
- ☐ Act in a way I respect

My own kind of care - something these lists have missed:

My low-energy plan

CHAPTER REFLECTION

Early signs that I am running low:

When I have capacity	My smaller version
A meal or food plan:	
A connection or support:	
A practical task:	
Something comforting or enjoyable:	

One thing I can ask someone else to help with:

MY NEXT STEP

Goal	Is my aim restoration, support, pleasure or something else? _____
Now	What resources do I really have today? _____
Options	What can wait, become smaller or be shared? _____
Next step	My act of care is _____. I will ask _____ for _____.

Care does not have to earn its place through productivity. A difficult day is information about what you need, not a failed self-care performance.

Food & body care

Consider: What would I like food to support in my life?

Food gives us energy and nutrients. It can also hold pleasure, culture, comfort, routine and connection. Calories describe energy; they do not describe the whole nutritional value or meaning of a meal.

Protein, fibre, vitamins, minerals, hydration and enough food all matter. So do access, appetite, allergies and medical needs. A choice that is useful for someone else may be unsuitable for you.

A meal that supports me - and what helps it happen:

One pressure about food that I would like to question:

An ordinary day of eating

A food diary can use words instead of numbers. Describe what happened around eating: time, company, access, hunger, satisfaction and how you felt. Avoid marking a meal as a success or failure.

Time / meal	What I ate or drank	Context / appetite / satisfaction

When was eating easy? When did it feel rushed, delayed or complicated?

What do I notice about energy or mood without assuming food caused it?

Hunger and fullness signals can be affected by stress, medication, illness or a history of restrictive eating. A prescribed meal plan or individual support may be more useful than relying on appetite alone.

Build a satisfying meal

A flexible meal might bring together a protein food, a starchy or wholegrain food, vegetables or fruit, and suitable fats. This is a starting idea, not a rule that every plate must meet. Meals across the day can supply different things.

Part of a meal	Possibilities I enjoy
Protein	Beans, lentils, tofu, eggs, yoghurt, fish, meat or another suitable choice.
Energy + fibre	Potatoes, rice, bread, oats, pasta or other grains; wholegrain options where they suit me.
Colour + variety	Fresh, frozen or tinned vegetables and fruit. Choose what I can obtain and use.
Flavour + fats	Nuts, seeds, oils, spreads, sauces, herbs or spices that fit the meal and my needs.
Drink	Water or another suitable drink. Individual fluid advice comes first.

My easy meal: the ingredients, the time it takes and why I like it:

My low-energy or low-budget version:

What would a great leader do?

How would they make nourishment more available, rather than turn every meal into a test?

Make food easier

An obstacle I can influence

An empty cupboard, no lunch break planned, or a recipe that asks more energy than I have.

An obstacle needing support

Limited money, inaccessible shops, swallowing difficulties, pain, caring demands or a prescribed diet.

The difficult moment in my food routine is usually…

What is the smallest useful change to the situation?

When energy is available	When energy is low
A food I can prepare / obtain:	A food I can keep ready:
A person or service I could ask:	An easier way to eat enough:

A food I enjoy and want to keep room for:

Fullness & satisfaction

Protein can support fullness. Fibre, food volume, flavour, texture and eating enough also matter. A meal that suits one person may leave another hungry. Notice your pattern across a few ordinary meals before making a sweeping conclusion.

In a small controlled study, 20 adults ate freely from an ultra-processed menu and an unprocessed menu for two weeks each. They ate about 500 more calories a day on the ultra-processed menu, gaining an average 0.9 kg while losing 0.9 kg on the other menu. The gain was not all fat. This study compared whole menus; it did not test the brands in this journal or predict an individual result.

Try looking at the whole meal

At home or with a takeaway, consider the food you enjoy, a protein source, available vegetables and portions that fit your appetite and circumstances. Sauces, sides and drinks are part of the meal too. Convenience foods can be useful; processing alone is not a complete guide to nutritional value.

A meal that usually keeps me satisfied:

What could I add or change in a meal that leaves me wanting more?

Choose your approach

Route	What I will notice
Food awareness	Regular access to meals, satisfaction, variety, enjoyment and practical support.
Optional food logging	A brief record of portions or nutrients to answer a specific question.
Optional energy planning	Rough estimates for an appropriate adult goal, with the limitations that follow.
Optional weighing	A longer-term pattern, only if the information is useful and emotionally manageable.

If numbers increase anxiety, guilt, repeated checking or rigid eating, use the awareness route and consider appropriate support. No body size requires you to complete the weight pages.

> **Before using the calculations**
> These estimates are for adults. Do not use them as self-directed targets for under-18s, pregnancy or breastfeeding. With low weight, unintended weight loss, an eating-disorder history, kidney disease, a prescribed diet or medicines that can cause low blood sugar, seek individual dietetic or clinical advice.

The question I want a tool to answer is…

My route for now, and how I will know the tool is no longer helping:

Energy balance

Your body uses energy to keep you alive, digest food and do everyday activities. Over time, the relationship between intake and expenditure influences body weight. It is not a simple explanation of your health, habits or worth.

Pattern over time	What it means
Deficit	Taking in less energy than you use usually leads to weight loss.
Maintenance	Taking in roughly what you use tends to keep weight broadly stable, despite daily fluctuations.
Surplus	Taking in more energy than you use usually leads to weight gain. Extra food alone does not become extra muscle.

Estimates are starting points. Energy needs vary, food records have errors and requirements can change. Weight change also includes water and other tissues, so short-term scale changes do not measure fat change precisely.

If I have a weight-related goal, why does it matter to me?

What would tell me I am supporting my life while pursuing it?

Weight loss is not needed to use the body-check or reflection chapters. Muscle, energy, comfort, food enjoyment and everyday function can matter regardless of weight change.

Estimate energy needs

1 / Estimate resting energy

The Mifflin-St Jeor equation uses weight in kilograms, height in centimetres and age in years. It estimates resting energy; this is **not** a daily food target.

> **(10 × kg) + (6.25 × cm) − (5 × age)**
> Then -161 for the original female equation, or +5 for the original male equation.

Example: 75 kg, 170 cm, age 40.
(10 × 75) + (6.25 × 170) - (5 × 40) = 1,612.5.
Female equation: 1,451.5 kcal/day. Male equation: 1,617.5 kcal/day.

2 / Include the whole day

Rough factor	Usual activity
1.4	Mostly sitting, with little daily movement.
1.6	Mostly sitting, plus regular moderate activity.
1.8-2.0	More movement through the day and/or substantial training.

Example: 1,451.5 × 1.4 = 2,032.1, rounded to about 2,000 kcal/day.

My resting estimate: _____ × **factor:** _____ = _____ kcal/day.

Developed in adults aged 19-78; the original sex categories do not describe gender identity. The factors are simplified and can be inaccurate for an individual. For circumstances outside its scope, including hormone treatment, seek tailored advice. Unit help: pounds ÷ 2.2046 = kg; inches × 2.54 = cm.

Do it / NIDDK Body Weight Planner
Choose units and enter your starting details, activity and any appropriate weight goal and date. Review the estimates; goal and maintenance figures mean different things.
www.niddk.nih.gov/bwp

For adults aged 18+; not for pregnancy or breastfeeding. A result is not a prescription. Read Choose your approach · p. 19 for other reasons to seek individual advice first.

Understand an adjustment

For adults for whom fat loss is appropriate, a modest adjustment may be more manageable than a large cut. A practical starting example is 10-20% below an estimated maintenance level, often towards 10-15% first. Review wellbeing and a longer-term pattern before changing it.

Example goal	At 2,000 kcal maintenance
10-20% deficit	1,600-1,800 kcal/day
Maintenance	About 2,000 kcal/day
5-10% surplus	2,100-2,200 kcal/day

A surplus is sometimes used alongside resistance training for muscle gain; it is not a recommendation that everyone should eat more. This workbook does not prescribe a training programme.

Arithmetic: a 15% deficit uses maintenance × 0.85: 2,000 × 0.85 = 1,700. Maintenance uses 1.00; a 5% surplus uses 1.05.

If appropriate for me: my estimate _____ × multiplier _____ = _____.

How will I check energy, hunger, concentration and everyday capability?

Protein

Protein supplies building blocks for body tissues. Adequate protein and appropriate resistance training can help preserve muscle during weight loss. Protein does not replace enough food overall, and powders are optional.

Context	Daily guide
Basic adult reference	0.83 g per kg body weight
Healthy older adults, usually 65+	1.0-1.2 g/kg
Healthy adults exercising regularly	1.4-2.0 g/kg

These describe different contexts; do not add them together. A higher-end figure sometimes used with resistance training is 1 g per pound, about 2.2 g/kg. It is not a minimum everyone needs. Many people do well with less.

Example: 75 kg × 1.6 = 120 g/day. At 2.2 g/kg, the same weight gives 165 g/day. Count protein from all foods, and keep fruit, vegetables, fibre, carbohydrates and suitable fats in the picture.

Protein foods I enjoy and can obtain:

Optional calculation, if suitable: _____ kg × _____ g/kg = _____ g/day.

With kidney disease, frailty or very high or low body weight, ask for help choosing a target. Sources: EFSA adult reference; PROT-AGE older-adult guidance; ISSN position stand; Morton and colleagues.

PROTEIN USES MORE ENERGY DURING PROCESSING

20-30%	5-10%	0-3%
protein	**carbohydrate**	**fat**

Approximate energy used processing each nutrient. Log full food calories; do not subtract a discount.

Food logging

Use a food record only if it answers a useful question. Words about meals and appetite are an alternative to numbers.

1. **Choose the portion**
   Check the food or package and the portion you ate. Match raw weights with raw-food entries and cooked weights with cooked entries.

2. **Record it**
   In MyFitnessPal, use Log / Food, the plus button or Add Food. Search the food or brand, check the entry and set the portion. Include relevant drinks, oils and sauces.

3. **Read what you recorded**
   Look for the daily food-energy total and protein in grams. Menus and access can vary. A food log records intake; an app target is a separate estimate.

Without an app: protein per 100 g × portion in grams ÷ 100 = protein in that portion. For example, 8 g per 100 g × 150 g ÷ 100 = 12 g. The same calculation works for kcal. Use an entry that matches preparation.

Do it / MyFitnessPal food logging help
Official setup steps. Food logging is available without Premium; other features and access may differ.
support.myfitnesspal.com · Food logging help

What did the record help me notice?

Understand it: Food & body care · p. 14 and Protein · p. 23. If logging increases anxiety, guilt, meal skipping or rigid rules, pause and seek suitable support.

Read a food label

A per-100 g label helps compare products at the same weight. A per-serving label helps describe the portion you actually eat. Check both. A lower calorie value is not automatically the better choice for your needs.

Example products	Per 100 g: kcal / protein
Brennans Be Good Wholemeal / Family Pan	212 / 9.4 g • 248 / 10.4 g
Arla Skyr Natural / Glenisk Organic Greek Style Natural yoghurt	61 / 10 g • 121 / 2.8 g
Tesco Roast Chicken Breast Pieces / Tesco Breaded Chicken Goujons	115 / 23.8 g • 255 / 13.1 g
Flora Lighter / Lurpak Slightly Salted Butter	252 kcal • 739 kcal

Illustrative portions: two bread slices may be 120 versus 198 kcal, but these portions weigh about 57.2 g versus 80 g. At 200 g, the yoghurts are 122 kcal / 20 g protein versus 242 kcal / 5.6 g. At 150 g, the chicken examples are about 173 kcal / 35.7 g versus 383 kcal / 19.7 g. At 10 g, the spreads are about 25 versus 74 kcal.

These are examples from product labels in the source material, not a ranking or current-price comparison. Recipes and labels can change. Chicken product identities and retailer links appear in Sources. Use your own pack for a purchase or food log.

The combined illustration: choosing all four lower-calorie portions changes their total by about 460 kcal. This is not a full-day menu, a recommendation to swap all four, or a prediction of weight change.

Two foods I buy: what differs per portion and what matters to me?

Which differences matter for my taste, needs, budget and goal?

What the scales can tell me

Body weight includes fat, muscle, water, bones, organs and the contents of your digestive system. A change on the scales cannot tell you which part changed. Salt, stored carbohydrate, menstrual-cycle changes, meals, drinks and the timing of a reading can affect the number.

> **500 ml of water weighs about 0.5 kg**
> That is the weight of the water, not body fat. Keep drinking normally. A weigh-in is never a reason to restrict fluids.

Home body-fat scales estimate body composition. Do not treat a daily “fat gained” or “muscle lost” message as a precise finding. Scale weight also cannot tell you how strong, fit or healthy you are.

When it helps

It answers a useful question about a longer-term trend without deciding the rest of your day.

When it takes over

You repeatedly check, feel judged by the reading, skip food or change your day to compensate.

What would I want weighing to help me understand?

My choice: use the scales / take a break / no home weighing. Why?

Follow an existing healthcare monitoring plan. For under-18s, pregnancy or an eating-disorder history, seek individual advice rather than planning weight change alone.

Record a useful trend

Use the same scales on a firm, level floor, at a similar time and in similar clothing. Morning after the toilet and before breakfast is one practical option. Ask for an accessible alternative if standing is difficult.

Weekly is a simple choice. Daily readings are optional if they feel neutral and you use an average; less often or no weighing is fine. Do not weigh more than once a day unless your care team advises it. Keep food and fluids normal.

Date	Weight / unit	Useful context

If using several readings

Add them, then divide by how many you recorded. Example: 75.0 + 75.4 + 74.8 + 75.2 = 300.4 kg. Divide by 4 = 75.1 kg. A later set of 74.9, 74.5, 74.8 and 75.4 averages 74.9 kg: the final reading rose, but the average fell.

These are example numbers, not target weights. Keep units, timing and frequency similar when comparing averages. Include higher readings too. Look across several weeks; even a trend does not precisely measure fat change.

My next step: keep / adjust / pause tracking / seek support. Because…

Food: my next step

CHAPTER REFLECTION

What have I learnt about food, my body or my circumstances?

With or without measurements	What matters to me
Everyday life	Energy, concentration, comfortable clothes, easier tasks or recovery.
Food experience	More satisfying meals, variety, enjoyment or less pressure.
Practical care	A workable shopping routine, enough food or useful support.

MY NEXT STEP

Goal	Does my original goal still fit? _____
Now	What did I learn about my actual needs and resources? _____
Options	Which option supports me without adding unnecessary pressure? _____
Next step	I will try _____. My smaller version is _____.

Screen time & attention

Consider: What do I want more attention or time for?

A total number of hours cannot tell you the whole story. Look at what you do, when you do it, how it leaves you feeling and what it may replace: sleep, movement, concentration or being with someone.

> **Ask about the experience as well as the hours.**
> After using this app, do I feel connected, informed, rested, distracted, tense or something else?

In a 2025 randomised trial, blocking mobile internet on phones for two weeks improved measured attention, wellbeing and mental-health outcomes. Calls, texts and internet on other devices remained available. This was a specific intervention, with adherence limits; it does not establish a universal daily screen-time limit or prove that a particular device will help you.

When I reach for my phone without planning to, what am I looking for?

What would I like to give ten reclaimed minutes to?

Research: Castelo and colleagues (2025), PNAS Nexus. A personal experiment can reveal a pattern; it cannot establish the cause of a mental-health difficulty.

Protect my attention

CHAPTER REFLECTION

Free options first	What I could try
Reduce prompts	Turn off a few non-essential notifications.
Create a place	Put the phone away during one chosen activity.
Use built-in tools	Try app limits, focus settings or scheduled quiet time.
Plan the replacement	Keep a book, sketchpad, activity or person in mind.

Optional tool / Brick
A physical device and app that block selected apps. Check current phone compatibility and essential-access settings.
getbrick.com/

My experiment: during _____ I will _____.

MY NEXT STEP

Goal	Did this make room for something that matters? _____
Now	What happened to my attention or mood? What else was happening? _____
Options	Which free or paid option fits my actual circumstances? _____
Next step	I will keep/change _____ and carry it into _____.

What would a great leader do?

How would they protect attention while staying available for what matters?

Meditation

Consider: What would I like from a short pause?

Meditation includes practices that train attention or awareness. Mindfulness-based approaches may help some people with stress and other symptoms, but results vary and research has limitations. You do not have to feel calm afterwards for your observation to be useful.

A simple one-minute experiment

1. Sit or stand comfortably. Your eyes can stay open.
2. Choose an anchor: a sound, your feet on the floor or your natural breathing.
3. Notice when attention wanders. Gently return.
4. Finish by looking around and noticing how you feel.

Before the pause I noticed _____. Afterwards _____.

If inward attention feels distressing, stop and reconnect with the room or choose another activity. Meditation does not replace needed healthcare. Source: NCCIH; this short activity is an original suggestion, not a validated treatment.

A guided pause

CHAPTER REFLECTION

davidji / five-minute guided meditation
Open the practice on davidji's own website. Notice your own response; there is no need to make yourself feel calm.
davidji.com/erase-anxiety-5min-gm/

Choose a safe moment to listen. Keep your eyes open if that feels better; stop if the practice increases distress.

What did I like or dislike about being guided?

Was I trying to achieve a particular feeling? What happened when I noticed that?

What would make a pause easier to include in my real day?

MY NEXT STEP

Goal	Does this kind of pause support my goal? _____
Now	What did I notice, including discomfort or no change? _____
Options	Try again, shorten it, use another anchor or skip it? _____
Next step	My next choice is _____. I can stop whenever I need.

Wheel of Life & GROW

Use this chapter when you want to explore your life-area snapshot or turn an intention into a realistic step. You can use the everyday pages without it.

	Meaning	A question to begin
G	Goal	What do I want to understand or change?
R	Reality	What is happening now, including my feelings, resources and limits?
O	Options	What could I try, adapt, ask for or leave alone?
W	Will / way forward	What am I willing to do, when, and with what support?

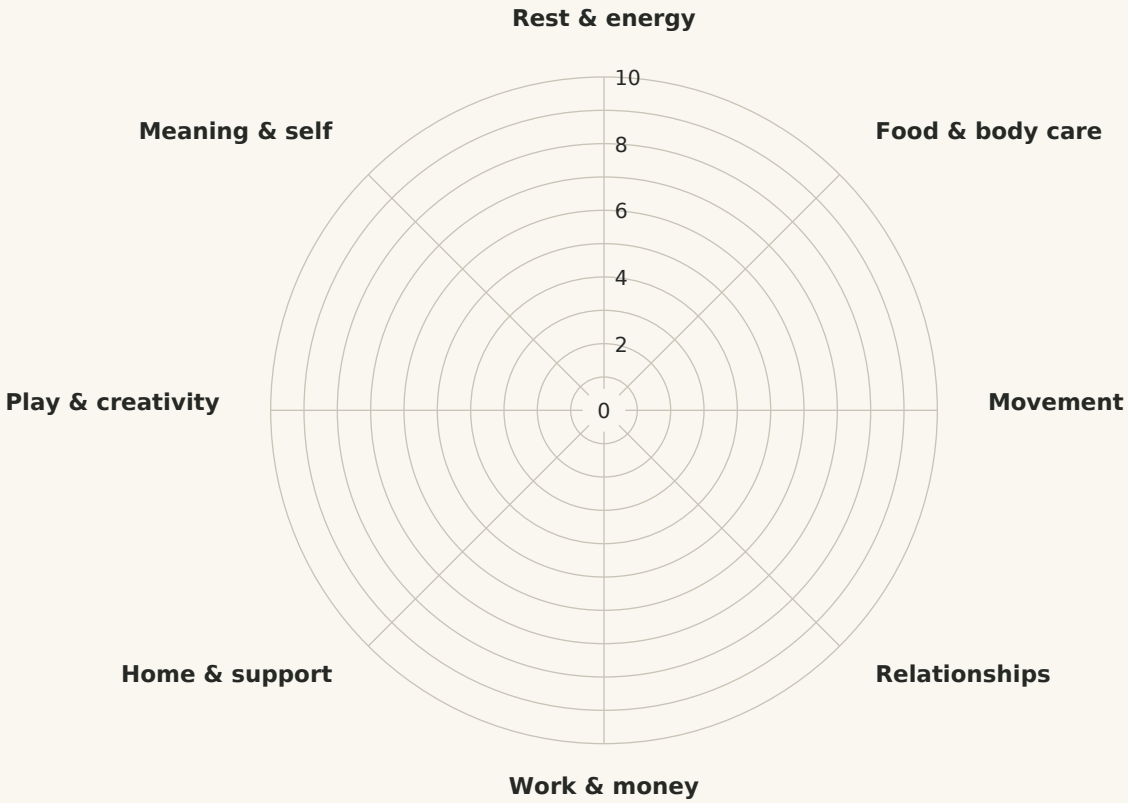
On the next page, transfer your scores from Your life right now (p. 3) if they still fit. Or score afresh and add a new date. The diagram is another way to see the same snapshot.

The change or question bringing me here:

GROW: Sir John Whitmore and colleagues. These explanations and questions are original adaptations. The Wheel of Life is a separate reflection tool.

My Wheel of Life

Score each area from 0 to 10: 0 = not working for me; 10 = working very well. Mark each spoke and join the points. Rename any area that does not fit.



Date: _____ The area I choose to focus on - and why:

The scores are personal judgements, not a test. The lowest score need not come first, and a balanced life need not form a perfect circle.

Behind the scores

Area	A useful question	A few words
Rest + energy	Where do I recover? What keeps using energy after I have run out?	
Food + body care	Do my everyday choices support me? What feels caring or pressured?	
Movement + strength	What would I like to do more comfortably or confidently?	
Relationships	Where can I be myself? Where do I feel connected or alone?	
Work + money	What matters about this area beyond other people's idea of success?	
Home + support	What makes daily life easier? What practical support is missing?	
Play + creativity	What do I enjoy without needing to be productive or impressive?	
Meaning + self	Which parts of life feel chosen? Which feel inherited or expected?	

Which strength elsewhere in my life could help this area?

Rename categories freely: parenting, faith, friendships, caring, study, community, access or something else. An overlapping area is useful if it helps you see your own life.

My first GROW

CHAPTER REFLECTION

G / The everyday change I want, and why it matters:

R / My current pattern, resources and limits:

O / Three possibilities, including a smaller or supported version:

W / I will try _____ when _____.

If this gets in the way: _____ I will: _____

Support: _____ Willingness: ____ / 10

Review point: _____

Example: protect ten minutes after dinner for a short walk or quiet pause; ask someone to share one evening task. Keep a step that suits your circumstances.

Fitness checks

These checks record how a task goes on a particular day. Choose one or two relevant to your life. Established tests and workbook adaptations are labelled; they are not a diagnosis or an exercise programme.

Everyday question	Possible check
Getting up from a chair	Chair stand
Steadiness / getting around	Balance / Timed Up and Go
Stamina	Walking / seated marching
Pushing / calf use / trunk support	Wall or counter task / calf raise

My goal: what would I like my body to do more comfortably or confidently?

The checks I choose, and any support I need:

Before a body check

Use stable equipment, a clear non-slip space, suitable footwear and any usual walking aid. Have a helper where the method asks for one. Warm up briefly with comfortable familiar movement; rest between checks until recovered. Avoid testing when unwell or unusually fatigued.

> **Stop rather than push through**
> Stop for pain, dizziness, loss of control or feeling unwell. Chest pain, fainting, severe unusual breathlessness or new weakness need urgent medical help. In Ireland, call 112 or 999; elsewhere use your local emergency number.

Get individual advice first after recent surgery or injury, unexplained fainting, repeated falls, or with a condition that affects safe activity. If balance is uncertain, work with a qualified professional. Never arrange an unsupported challenge simply to obtain a score.

Make a later comparison fair

Use the same chair height, route, surface, support, footwear, timing and movement rules. Record the version. A supported version can be useful, but its result belongs in a separate record. Rest, symptoms, practice and encouragement can all affect a result.

My space, equipment, helper and any advice I need:

What I will keep the same when I return:

Effort scale used here: 0 = resting; 10 = maximum effort. It describes your experience, not a target to reach. These adult reference tables do not apply to children.

The 30-second chair stand

You need: a firm armless chair against a wall, a timer and a helper to count. The standard setup uses a seat about 43 cm / 17 inches high. Measure and record the height.

1. Sit in the middle of the seat, feet flat, back upright. Cross your wrists and place your hands on opposite shoulders, arms against your chest.

2. On “Go”, stand fully, then sit back down under control. Continue for 30 seconds.

3. Count full stands. Count the final rise if you are more than halfway up when time ends.

Practise two rises, then rest at least a minute and until recovered before one recorded attempt. Stop if control is lost or symptoms appear. Keep the practice and rest routine the same next time.

> **If you need your hands**
> Stop the standard test and record 0 under that test’s scoring rule. After resting, an assisted version may use a firm armchair or fixed support. Record that support and keep the assisted result separate.

Optional demonstration / CDC chair-stand test
youtu.be/qkV0UvjXgcs

Date / setup	Result / version	Comfort / effort / context

Chair height _____ cm. Rises _____. Effort 0-10 _____. What did I notice?

Chair-stand reference ranges

Ages 60-94: middle 50% of scores in the US Senior Fitness Test reference population. Use the standard no-hands chair-rise setup.

Age	Men	Women
60-64	14-19	12-17
65-69	12-18	11-16
70-74	12-17	10-15
75-79	11-17	10-15
80-84	10-15	9-14
85-89	8-14	8-13
90-94	7-12	4-11

Ages 20-59: rounded mean scores from independently living, apparently healthy Danish adults in the Copenhagen Sarcopenia Study. This used a 45 cm chair and counted fully completed stands: background context, not direct grades for the 43 cm check.

Age	Men	Women
20-29	26	27
30-39	28	27
40-49	26	25
50-59	23	22

These are the categories reported in the original studies. Health, body size, setup and practice affect comparison. The most useful personal comparison is often you with the same setup. A persistent decline or new difficulty deserves a clinical conversation. Sources: Jones & Rikli (2002); Suetta and colleagues (2019).

Standing balance

Setup: firm floor, fixed support within reach and someone able to guard you safely. If you are unsteady or have fallen, do this with a qualified professional. Use support to settle into a position; release it only when steady and start the timer.

Keep eyes open; arms may move. Stop timing at 10 seconds, if a foot moves, if the lifted foot touches down, or if you need support. Progress only after completing 10 seconds. Leave untried positions blank.

Position	First date ___	Later date ___
Feet together: side by side.	____ seconds	____ seconds
One foot slightly forward: instep touches the other big toe.	____ seconds	____ seconds
Heel to toe: heel directly in front of and touching the other toes.	____ seconds	____ seconds
One foot: lift the other foot clear of the floor.	____ seconds	____ seconds

Supported option: keep your hands on a fixed counter and hold a comfortable stance for up to 10 seconds. Record it as a separate observation, with the support described.

Optional demonstration / CDC four-stage balance test
youtu.be/VUq6IgQAVJM

Which foot was forward / on the floor? What support or difficulty did I notice?

Ten seconds is the limit of this test, not a complete measure of balance or a guarantee against falling. Source: CDC STEADI.

Timed Up and Go

> **Three metres. no rush.**
> Mark a line 3 metres from the front of a firm armchair, on a clear, level route.

You need: a timer, a helper, your usual footwear and walking aid. The helper explains the route and stays nearby.

1. Start seated, back against the chair.

2. On “Go”, stand, walk to the line, turn, walk back and sit down.

3. Time from “Go” until seated again. Use your normal pace throughout.

Practise once, rest until recovered, then record one timed attempt. Use the same practice routine next time. Record the chair height, turn direction, walking aid and whether you used your arms.

Optional demonstration / CDC Timed Up and Go
youtu.be/tNay64Mab78

Date / setup	Result / version	Comfort / effort / context

Time _____ seconds. Chair height _____ cm. Did I feel steady?

What did I notice about standing, walking or turning?

Keep the same setup. A faster result achieved by rushing is a different performance. If standing or walking is unsuitable, use a separate seated observation. A time alone cannot diagnose a mobility problem. Source: CDC STEADI.

Comfortable walking

You need: a safe, level route with a known length, a timer, usual footwear and aid, and a helper if needed. Choose a duration that is already appropriate for you. Keep it the same for comparison.

1. Begin at a comfortable, purposeful pace at which you can speak in sentences.

2. Walk for the chosen time. If you rest, keep the clock running and note the rest.

3. Count completed lengths and measure the final part. Record distance, effort and symptoms.

Example: 12 lengths of a 20-metre route, plus 8 metres, gives (12 × 20) + 8 = 248 metres. A route with frequent turns differs from a longer straight route.

Optional demonstration / Talk-test / pace guide - movement guidance, not this exact check
youtu.be/5dt-SRPbEll

Date	Duration / route	Distance / rests / effort

What did I notice during the task and as I recovered?

Do not aim for maximum effort. Use the stop guidance at the start of this chapter. This adapted check has no diagnostic cut-off or universal target distance.

Seated marching

Use a stable chair, feet on the floor and enough space. Choose 2 minutes, or a shorter duration in advance if that is more appropriate. A seated task is not automatically suitable for every condition; use individual advice when needed.

1. Sit upright with support as needed. Practise a few comfortable alternating knee lifts.

2. Start the timer and march at an easy, sustainable rhythm. Use a comfortable repeatable knee height.

3. Count each lift of the right knee. Keep the timer running during rests. Stop for symptoms or loss of comfortable control.

Record the chair, support, duration, knee height, count, rests and effort. If you change the height or duration, label it as a new version. Do not compare the count with walking distance.

Optional demonstration / Seated marching movement
youtu.be/D_AEO1YriE4

Date / setup	Result / version	Comfort / effort / context

Duration _____. Right-knee lifts _____. Effort 0-10 _____.

What made this comfortable or difficult today?

This is a workbook observation, not a standardised fitness diagnosis. The demonstration shows the movement; use the timing and recording rules on this page.

Wall or counter press

Use a clear, non-slip space and a wall or fixed counter that will not move. Begin with the easier wall angle. Record the surface height, distance of feet from it and hand spacing.

1. Place hands around shoulder width, body in a comfortable straight line. Keep feet secure.

2. Bend your elbows to lower under control for about 2 seconds, then press back over about 1 second. Breathe normally.

3. Count controlled repetitions, up to a maximum of 20. Stop earlier if posture changes, control is lost or symptoms appear.

Practise 2-3 repetitions, then rest for at least a minute and until recovered before one recorded attempt. Keep the same depth, surface, pace and setup for a later comparison. If wrists or shoulders hurt, stop and choose another suitable observation with advice if needed.

Optional demonstration / Wall press-up / plank movement
youtu.be/KKlB_uiX3Dc

Date / setup	Result / version	Comfort / effort / context

Repetitions _____. Surface / foot distance _____. Effort _____.

What did I learn that matters in ordinary life?

This adapted check has no clinical pass mark. Twenty is a ceiling for recording, not a requirement or a training prescription.

Calf raises

Stand on a level, non-slip floor beside a fixed support. Use light fingertip contact for balance, rather than pulling yourself up. Keep your chosen footwear the same each time.

1. With both feet on the floor, rise onto the balls of your feet over about 1 second.

2. Lower over about 1 second, keeping the movement controlled.

3. Count repetitions to a maximum of 25. Keep a consistent heel height. Stop if height falls, you rely on your arms, control changes or symptoms appear.

Practise 2-3 comfortable raises and rest for at least a minute and until recovered before recording. A seated heel raise is a separate supported option. A single-leg version is optional only if already familiar and suitable for you; record it separately, with at least 2 minutes between sides and longer if not recovered.

Optional demonstration / Calf-raise movement
youtu.be/cqDMYUaIXvw

Date / setup	Result / version	Comfort / effort / context

Both legs / one leg? Repetitions _____. Support _____. Effort _____.

How did the movement feel on each side?

Do not use a stair edge for this check. The cap of 25 is a recording limit, not a target. A new asymmetry, pain or persistent difficulty can be discussed with a clinician.

Direction & response

Side-steps / 30 seconds

Mark two lines 1 metre apart on a clear, non-slip floor. Practise both directions. On “Go”, side-step from one line to the other without crossing your feet. Count a completed length when both feet cross the destination line. Continue for 30 seconds, stopping for loss of balance, symptoms or control. Record completed lengths and any support.

Optional demonstration / Side-step movement - not the cue task below
youtu.be/1QMX5eWWC3Y

Optional cue task / 10 instructions

With a helper, mark a target about 30 cm to each side of a centre point. The helper gives 10 “left” or “right” cues, 5 of each, in a newly mixed order, around 4 seconds apart. Step to the named target and return to centre. Record correct responses and balance errors. Stop if unsteady or unable to return before the next cue without rushing. Hearing, understanding and practice can affect this task; it is not a laboratory reaction-time test.

Seated alternative: tap left and right targets on a table to the same cue pattern, using a comfortable reach. Label and compare this separately. Supported side-stepping along a fixed counter can instead be observed without timing speed.

Task / setup	First record	Later record
Side-steps: lengths / support		
Cue task: correct / errors / standing or seated		

What helped me respond? Did rushing change the task?

These are original adapted observations, not validated clinical scores. Leave the standing options out if balance is uncertain.

Trunk support

Choose a safe surface and an easy body angle. Place hands on the wall or counter and step back a little, keeping a comfortable straight line through your body. Note surface height, foot position and hand spacing.

1. Try a short practice hold. Rest at least 1 minute and until recovered.

2. Start timing once settled. Breathe normally.

3. Stop at 30 seconds or earlier if posture changes, you hold your breath, wrists become uncomfortable, or other symptoms appear.

Record the time and setup. A different angle changes the task. More time is not automatically more useful once you reach the recording cap.

Optional demonstration / Wall plank / press-up movement
youtu.be/KKlB_uiX3Dc

Date / setup	Result / version	Comfort / effort / context

Hold _____ seconds. Setup _____. Effort _____.

A separate seated observation

If a wall or counter task is unsuitable, sit in a stable chair and make a small comfortable reach within your base of support. Keep feet supported; stop if unsteady. Describe control and comfort rather than assigning a comparable hold score.

These adapted observations cannot diagnose “core weakness”. Use the result as one clue about the particular task.

My fitness comparison

CHAPTER REFLECTION

Check / version	First result	Later result	Setup / effort / notes
Chair stand + chair height			
Balance			
Timed Up and Go			
Walking / seated marching			
Wall / counter press			
Calf raise			
Side-step / cue response			
Trunk-support hold			

MY NEXT STEP

Goal	The everyday goal this informs: _____
Now	What changed in the result or experience? _____
Options	Repeat, adapt, pause or seek advice? _____
Next step	My next step / when / support: _____

Compare the same method, setup and support. A new version starts a separate record; an easier-feeling task can matter even when the number stays the same.

Mobility & comfort

Consider: What everyday movement would I like to understand?

These checks describe movement and comfort. They cannot tell you why something hurts or diagnose a joint, nerve or muscle problem. Body proportions, chair height and technique affect what you see.

Use a stable seat or fixed support, move slowly and breathe normally. Stay within a comfortable range. Do not bounce, force, pull on your head or use someone else to push you farther. Read the specific instructions and stop for pain, dizziness or new symptoms.

> **One practice. two observations.**
> For the simple movement checks, try one gentle practice then observe twice, unless the page gives a different method. Record the side and version. There is no need to repeat until you get a “better” result.

The movement I choose and the ordinary task it connects with:

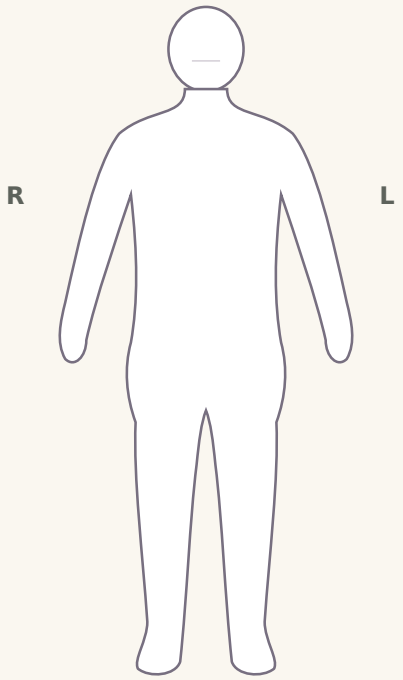
Follow clinical advice after injury or surgery. New weakness, major loss of movement or neurological symptoms need assessment; new bladder/bowel disturbance with back pain or saddle numbness needs urgent help. Choose support rather than using this book to investigate symptoms alone.

My body map

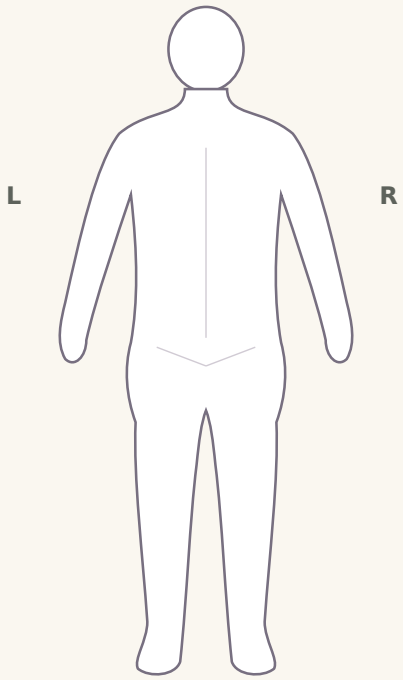
Mark an area where you notice pain, tension or discomfort. Add a number if you want to connect more than one mark with your notes.

R and L mean your right and left. Mark on paper or use your PDF app’s drawing tool.

FRONT



BACK



Date / time

Location / mark number

Severity · /10

0 = no discomfort; 10 = worst imaginable. Describe your own experience.

When does it occur?

What has changed since last time?

What seems to ease it?

What seems to aggravate it?

This map records an experience, not its cause. Do not provoke symptoms to fill it in. New, severe or persistent symptoms deserve appropriate assessment; see the guidance on Mobility & comfort · p. 50.

Neck turns

Sit or stand securely, head facing forward and shoulders relaxed. Keep the chin level. Slowly turn to look over one shoulder as far as is comfortable, return to centre, then try the other side. Keep shoulders facing forward and do not pull your head with your hands.

Make one gentle practice and two observations to each side. Describe what you can comfortably see or how the movement feels. You do not need a degree measurement or a perfectly equal range.

Optional demonstration / Neck rotation movement
youtu.be/UbHEH6t_OJQ

Side	What I noticed today	Later observation
Left		
Right		

When is looking around relevant in my day?

What would I like to ask a clinician if this remains uncomfortable?

Stop for pain, dizziness, visual changes, tingling or other new symptoms and seek appropriate advice. Do not repeatedly provoke symptoms to describe them. General neck advice: NHS inform.

Shoulder reach

Sit or stand securely. With thumb pointing up, slowly raise one arm forwards and upwards within a comfortable range, then lower. Let the shoulder blade move naturally; avoid leaning backwards to make the reach appear larger. Try each side separately.

Use one practice and two observations per side. Describe the highest comfortable position and the sensation. Do not force a painful arc or compare yourself with a picture of someone else.

Supported alternative

Seated at a stable table, rest the forearm on it and slide the hand forwards within a comfortable range, allowing a gentle lean from the hips if suitable. Record this as a supported version, not the same test as lifting your arm.

Optional demonstration / Shoulder movement
youtu.be/0u8IiwhIPT0

Date / setup	Result / version	Comfort / effort / context

Left / right / version: where did the movement feel easy or limited?

An everyday reach I would like to make more comfortably:

Keep the version the same next time. Persistent pain, new weakness or a sudden loss of movement deserves individual assessment.

Seated trunk turn

Sit on a stable chair, feet supported and hips facing forwards. Cross your arms loosely or keep them comfortable. Slowly turn your chest to one side, letting your head follow, then return to centre. Try the other side. Keep the pelvis facing forwards rather than sliding around the seat.

Use one practice and two observations per side. Stay within comfort. Avoid using your hands on the chair to pull yourself further or holding your breath.

Optional demonstration / Seated trunk rotation
youtu.be/JEDRfMbURRU

Side	Comfort / movement today	A later observation
Left		
Right		

Where does this movement show up in my daily life?

Was I observing, or trying to prove something? What would make the check gentler?

The observation cannot identify the source of back pain. If turning produces pain or neurological symptoms, stop and seek suitable advice.

Hip & knee movement

Thigh lift

Sit upright with feet supported. Lift one knee a little towards your chest, within a comfortable range, without leaning sharply backwards or pulling it with your hands. Lower slowly. Try each side: one practice, then two observations.

Optional demonstration / Seated thigh-lift movement
youtu.be/1m72e65YZtM

Knee bend and straighten

While seated, slowly straighten one knee as far as comfortable, then bend it again, keeping the thigh supported. Do not lock or force the joint. Use one practice and two observations per side.

Optional demonstration / Seated knee movement
youtu.be/HB0hHrXwImI

Movement	Left: comfort / control	Right: comfort / control
Thigh lift		
Knee bend / straighten		

Which everyday task connects with what I noticed?

A smaller range can reflect many things; do not infer a diagnosis. Follow any limits given after joint surgery or injury.

Chair sit-and-reach

Use a firm chair against a wall and a ruler; a helper can measure. Sit securely near the front without risking sliding off. Keep one foot flat; extend the other leg with heel down and toes pointing up. Line up your middle fingertips, stack your hands and hinge gently forwards from the hips towards that foot. Stop at a comfortable stretch, without bouncing.

Use two gentle practices, then two measured attempts per side. Hold the comfortable end position about 2 seconds. Measure fingertips relative to the toe: short = minus, at the toe = 0, beyond = plus. Record the best comfortable attempt, chair height and side.

Optional demonstration / Chair sit-and-reach movement
youtu.be/JYcO4vC3E_4

Side / date	Attempt 1 / cm	Attempt 2 / cm	Best comfortable
Left			
Right			

Example: -6 cm to -2 cm means reaching 4 cm farther on this task. It does not prove that a muscle became 4 cm longer. Arm, leg and trunk proportions and chair height affect the result.

Chair height _____. What did I feel, and where?

If extending the leg or leaning forwards is unsuitable, leave this out and use the knee observation instead. Do not force stretching through pain, tingling or numbness. Source: Jones and colleagues (1998).

Knee-to-wall

Face a wall with a fixed support available. Place one foot pointing forwards. Bend the knee towards the wall over the middle toes, keeping the heel and whole foot on the floor. Move the foot back only as far as the knee can comfortably touch the wall without lifting the heel or twisting the foot.

After a practice, make three comfortable measurements on each side. Measure the distance from the big toe to the wall. Record the best comfortable distance, side, footwear and support. A helper can make measuring easier.

Optional demonstration / Knee-to-wall movement
youtu.be/kbzYML05Vac

Side / date	1 / cm	2 / cm	3 / cm	Best
Left				
Right				

Footwear / support / comfort / anything that changed the setup:

Seated alternative: keep the heel down and lift the front of the foot. Note ease and control, not a knee-to-wall distance.

Research suggests a change around 2 cm can exceed measurement error in some clinical settings. That is not a universal “normal” range or proof that a home measurement has changed meaningfully.

Do not push through ankle pain or move to an unstable surface. This task observes weight-bearing ankle movement; it does not diagnose its limits. Source: Powden, Hoch & Hoch (2015).

Elbows, wrists & hands

Observe gently	How to try it
Elbow	Bend and straighten within comfort. With elbow near your side, turn the palm up and down without forcing.
Wrist	Support the forearm. Move the hand up and down slowly within comfort; keep the movement at the wrist.
Hand + thumb	Open and close the hand comfortably. Touch the thumb to each fingertip if suitable; avoid squeezing hard.

Use one practice and two observations per movement and side. Notice comfort, control and any relevant difference. Leave out movements limited by recent injury, surgery or clinical advice.

Optional demonstration / Elbow movement
youtu.be/LDwUepsdA04

Optional demonstration / Wrist movement
youtu.be/dtwfwjMKWz4

Optional demonstration / Thumb / finger movement
youtu.be/9bb6cLmKf1U

What I noticed on the left and right:

An everyday task - dressing, carrying, writing or another - that matters to me:

Pain, numbness or new loss of strength needs appropriate advice. Comparing sides is an observation, not a diagnosis.

Feet & ankles

Sit securely with both feet supported. Gently lift the heels while keeping the front of the feet down, then lower. Next, keep heels down and lift the front of the feet and toes within comfort, then lower. Use one practice and two observations, without pushing for height.

Optional demonstration / Seated foot movement
youtu.be/JegXz_XPgwk

Movement	Left: comfort / control	Right: comfort / control
Heel lift		
Toe / forefoot lift		

Footwear / seat / support used:

What does this tell me about the movement itself? What remains unknown?

What would a great leader do?

Would they value a careful observation more than a bigger number? How could I apply that here?

This is separate from the standing calf-raise check. Persistent pain, numbness, swelling or a new movement difficulty deserves clinical advice.

My movement comparison

CHAPTER REFLECTION

Movement / version	First: L / R	Later: L / R	Setup / comfort
Neck turns			
Shoulder reach / table slide			
Trunk turns			
Thigh lift / knee			
Seated reach / cm			
Knee-to-wall / cm			
Elbow / wrist / hand			
Heel / toe lifts			
My ordinary task			

MY NEXT STEP

Goal	The everyday movement I want to support: _____
Now	What changed in comfort, control or circumstances? _____
Options	Adapt, observe later or discuss a symptom? _____
Next step	My next step / support / review point: _____

Use the same position, equipment and practice routine. A home observation cannot identify the cause of pain.

Review & continue

Look back at Your life right now (p. 3) and any daily or weekly notes. Use this wider review after exploring a topic or making a change; it is not another weekly task.

Area	What has changed?	What may explain it?
Sleep / rest		
Food / appetite / enjoyment		
Movement / comfort / capability		
Attention / screens		
Energy / mood / support		

What is working well enough to keep?

My next direction

CHAPTER REFLECTION

Keep	Adapt	Let go

MY NEXT STEP

Goal	My priority now: _____
Now	The capacity and resources I actually have: _____
Options	Options, including help or rest: _____
Next step	My next step / when / review point: _____

What would a great leader do?

Return to the daily check-in (p. 2) or weekly reflection (p. 6) when useful. If you already keep a separate tracker, there is no need to copy its daily figures here.

Food & body care

Food guidance
WHO: Healthy diet; NICE NG246: Physical activity and diet. General guidance; the percentage adjustments here are planning examples, not a universal NICE target.

Energy estimates
Mifflin et al. (1990), resting-energy equation, *AJCN, 51*, 241-247 (PMID 2305711). Activity factors are simplified estimates. NIDDK Body Weight Planner offers an alternative calculation.

Fullness and food intake
Hall et al. (2019), ultra-processed versus unprocessed menus, *Cell Metabolism, 30*, 67-77.e3. doi:10.1016/j.cmet.2019.05.008. The small, short trial compared whole diets, not individual brands.

Protein reference amounts
EFSA (2012): adult protein reference values; Bauer et al. (2013), PROT-AGE (doi:10.1016/j.jamda.2013.05.021); Jäger et al. (2017), ISSN position stand (doi:10.1186/s12970-017-0177-8); Morton et al. (2018), resistance-training review (PMID 28698222). These concern different populations and purposes.

Energy used in processing nutrients
National Academies (2023), *Dietary Reference Intakes for Energy*, chapter 6, drawing on Westerterp (2004). Do not subtract these percentages from a food log.

Product examples
Label sources: Brennans Be Good Wholemeal and Family Pan; Arla Skyr Natural; Glenisk Organic Greek Style Natural; Tesco Roast Chicken Breast Pieces and Breaded Chicken Goujons; Flora Lighter; Lurpak Slightly Salted. Values are illustrations: check the current pack and the preparation method.

Optional logging help
MyFitnessPal: How to log food to your diary explains the app steps if you choose to use it. Food notes without numbers are also useful.

Movement & everyday habits

Established test methods
CDC STEADI assessment sheets: 30-second chair stand, four-stage balance and Timed Up and Go. Match the written setup when interpreting a result.

Chair-stand reference figures
Jones, Rikli & Beam (1999), chair-stand study (PMID 10380242); Jones & Rikli (2002), Measuring functional fitness of older adults; Suetta et al. (2019), Copenhagen Sarcopenia Study (doi:10.1002/jcsm.12477). The younger-adult figures use a different chair and completion rule.

Mobility methods
Jones et al. (1998), chair sit-and-reach (PMID 9864752); Powden et al. (2015), weight-bearing lunge reliability (PMID 25704110). Neither provides a universal pass/fail score for every home movement check.

Adaptations and demonstrations
The adapted activities are observations, not a validated clinical assessment. Demonstration links stay beside each activity; follow this journal's written timing and scoring where a video differs.

Sleep and meditation evidence
NHLBI: Healthy sleep habits; NCCIH: Meditation and mindfulness - effectiveness and safety. The brief pause in this journal is an original practice invitation, not a tested treatment protocol.

Screen-time study
Castelo et al. (2025), blocking mobile internet, *PNAS Nexus, 4*, pgaf017. doi:10.1093/pnasnexus/pgaf017. This two-week trial does not establish a universal safe screen-time limit or test the Brick product.

Step-count setup: Apple Health and Google Fit official help are linked with QR codes on p. 7. Step counts are estimates; a description of movement is an alternative.

Reflection, tracking & extras

GROW model credit
Whitmore, J. (2017), *Coaching for Performance*, 5th ed., Nicholas Brealey. GROW is associated with Whitmore and colleagues; the wording here is original. The Wheel of Life is a separate reflection tool.

Weighing: usefulness and emotional effects
Madigan et al. (2015), systematic review (PMCID PMC4546162); 2016 correction: 10.1186/s12966-016-0366-x. Steinberg et al. (2014), adult trial (PMCID PMC4157390); Pacanowski et al. (2015), young-person observational study (PMID 26566095). Different populations and methods; weighing remains optional.

Interpreting weight and body-composition readings
Hall et al. (2011), energy imbalance and body weight (PMCID PMC3880593); Kanellakis et al. (2023), menstrual-cycle changes (PMID 37395124); Frija-Masson et al. (2021), smart-scale accuracy (doi:10.2196/22487).

Optional practical guidance
NHS England: How to record your weight explains a consistent weighing setup if tracking serves a useful purpose for you.

A few useful extras

The links and QR codes sit beside the relevant chapters.

Sleep / The Matt Walker Podcast
The linked episode offers more detail on deep sleep for readers who want to explore the topic.

Meditation / davidji
The linked five-minute recording offers a voice to follow when you would prefer guidance.

Attention / Brick
An optional device and app to explore if you want a physical step between yourself and selected apps. Free built-in controls are also options.

Independent resources; no affiliation or endorsement is implied. Product details and web addresses can change.